



INPATIENT RECORD

Hospital No: 67046832	Visit No:
Name:	Age/Sex:
Doctor Name:	Specialty:
Date:	

BP Systolic: 100 mmHg	BP Diastolic: 60 mmHg	Pulse Rate: 74 bt/min
Respiration: 20 br/min	Temperature: 97.6 °F	Delivered Oxygen Flow Rate: 0
Saturation(Oxygen): 98 %	AVPU Conscious level: AVPU12120A	Mean Arterial Pressure-MAP: 73 mmHg
Hourly urine Output: 100 millilitres per hour		

IP Initial Assessment Status : Entered Score : 0 Patient Arrival Date : 23/03/1926 Patient Arrival Time : 21:15 Chief Complaints : Complaints of recurrent episodes of cold, mouth breathing. History of Presenting illness : Baby Zohra Rumani aged 13 years girl presented with complaints of recurrent episodes of cold, mouth breathing. She was evaluated for the same and was admitted for further management. Allergy : NOT KNOWN Past Medical History : NO COMORBIDITIES VACCINATION UPTO DATE NO DEVELOPMENTAL DELAY Past Surgical History : NOTHING SIGNIFICANT Family History : . Social / Personal History : . Menstrual History : . Previous Pregnancy : . Current medications : AS PER CHART Pulse /min : 74 Resp Rate/min : 20 Temp F : 97.6 BP mm Hg : 100/60 Room Air : Y GRBS mg/dL : . Pallor : NO Icterus : NO Edema : NO Clubbing : NO Cyanosis : NO Lymphadenopathy : NO Others : . CVS : S1 S2+ RS : B/L NVBS+ PA : SOFT NON TENDER CNS : CONSIIOUS
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Others : .
Provisional Diagnosis : ADENOTONSILLITIS
Care Plan : coablation adenotonsillectomy under general anaesthesia
Spo2 : 98
Diet Plan : .

Forms

Pain Assessments
Score: 2
Type of Assessment: Initial Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 2

Additional Pain Assessment Details

Type: Dull, Pulling
Duration: Irregular
Continuous: Variable
Time: With position changes
Onset: Gradual

Associated Symptoms: Decrease activity

Paediatric Braden Q Scale MHE
Status : Entered
Score : 21

SENSORY PERCEPTION - Ability to respond meaningfully to pressure-related discomfort : 4 - NO IMPAIRMENT – Responds to verbal commands. Has no sensory deficit which would limit ability to feel or voice pain or discomfort
MOISTURE - Degree to which skin is exposed to moisture : 4 - RARELY MOIST – Skin is usually dry; linen only requires changing at routine intervals
ACTIVITY - Degree of physical activity : 3 - WALKS OCCASIONALLY – Walks occasionally during day, but for very short distances, with or without assistance. Spends majority of each shift in bed or chair
MOBILITY - Ability to change and control body position : 4 - NO LIMITATIONS – Makes major and frequent changes in position without assistance
NUTRITION - Usual food intake pattern : 3 - ADEQUATE – Eats over half of most meals. Eats a total of 4 servings of protein (meat, dairy products) each day. Occasionally refuses a meal, but will usually take a supplement if offered - OR - is on a tube feeding or TPN
FRICTION AND SHEAR : 3 - POTENTIAL PROBLEM– Moves feebly or requires minimum assistance. During a move, skin probably slides to some extent against sheets, chair, restraints, or other devices. Maintains relatively good position in chair or bed most of the time but occasionally slides down
13 - 15: Moderate risk



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The Braden scale assesses a patients risk of developing a pressure ulcer by examining six criteria. with a higher score meaning a lower risk of developing a pressure ulcer and vice-versa.

9 or less: Very High Risk

10 - 12: High Risk

15 - 18: Low Risk

16 - 23: No Risk

Total Score : 25

TISSUE PERFUSION AND OXYGENATION : 3- Adequate:Nomotensive: Oxygen saturation may be < 95% or hemoglobin may be < 100 mg /Lr or capillary refill may be > 2 secs: serum PH is normal

Type of Assessment : Initial Assessment

ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL

Status : Entered

Score : 0

CATEGORIES

TOTAL SCORE : 0

COMMENTS : 0 - LOW RISK

No Risk - 0 : Y

Risk Assessment - Fall Paediatric (Humpty Dumpty)

Status : Entered

Score : 8 Low Risk Humpty Dumpty Score

Indication for assessment : Reassessment

Age : 13 years old and above - 1

Gender : Female - 1

Diagnosis:

Other diagnosis - 1

Cognitive Impairment : Orientated to own ability - 1

Environmental Factors : Patient placed in bed - 2

Response to Surgery/Sedation/Anaesthesia : More than 48 hours / None - 1

Medication Usage : Other medications / None - 1

NURSING INITIAL ASSESSMENT - PAEDIATRIC

Status : Entered

Score : 0

Informant : MRS. ZOIYA NAYEEM

Relationship : MOTHER

Reliability : Good,Fair

Past History : NIL

Allergies Known : UNKNOWN

Family/ Personal History : -

Immunization History : IMMUNIZATION DONE

Development Milestone : DONE

Temp : 97.6

Heart Rate : 74

Respiratory Rate : 20

Spo2 : 98



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

On room air : Y
Blood Pressure : 100/60
Pain Score : 2/10
Height : 138
Weight : 27.9
Icterus : NO
Pallor : NO
Head to Toe assessment (mention any abnormality) : DONE
Clinical Diagnosis : C/O COUGH AT NIGHT SINCE 1 MONTH
POSTED FOR COABLATION ADENOTONSILLECTOMY
Diet : NPO
Investigations : NIL
Current medications : NIL
Others : NIL
Doctors Order : NIL
Nursing Diagnosis : ACUTE PAIN RELATED TO ADENOID AND TONSIL INFLAMMATION AS
EVIDENCED BY PAIN SCALE SCORE 4/10
Nursing Care Plan : * Assess the condition of the patient
*Eliminate factors that precipitate pain.
* Monitor characteristics of pain (Location, severity using appropriate pain scale), frequency, precipitating factors
and relief factors.
* followed with physician orders
* Provide restful and quiet environment

Patient Arrival Time : 09:20

Surgical Site Infection Prevention Bundle
Booked procedure : ADENOTONSILLECTOMY (LENT)
Date of procedure : 24/03/2026
Pre-operative chlorhexidine bath supplied : Yes
Bath taken night prior to surgery : Yes
Bath taken on the day of surgery : Yes
Prophylactic antibiotic prescribed : No
Patient Acuity Tool
Stable patient 1 to 11
Moderate Risk Patient 12 to 22
Complex Patient 22 to 33
High Risk Patient >34

Total Score :17

Forms

Nurses Endorsment Checklist
Status : Entered
Shift : Transfers
Situation : Surgery, NON MLC, Transfer Out
Comment / Chief Complaints / Diagnosis : POSTED FOR ADENOTONSILECTOMY
Background
Allergies : No



Hospital No: 67046832

Visit No:

Name:

Doctor Name:

Remarks : UNKNOWN
Diet : Yes
Remarks : NPO
Previous Medications : No
Remarks : NIL
Old reports added to TRAK : No
Remarks : NIL
Consents : Yes
Remarks : SURGERY
Cross Consultation : No
Remarks : NIL
Procedures performed before transfer : No
Remarks : NIL
Vulnerable : Yes
Remarks : AGE BELOW 15 YEARS
Past Medical / Surgical History : NIL
Level of Consciousness : Oriented
GCS : 15/15
Vitals -TPR & BP : is stable : Yes
Pain Score : 2
On MET/PET List : No
Risk For FALL : No
Risk For Pressure Ulcer : No
Risk for Aspiration : No
Existence of Pressure Ulcer : No
Remarks : SKIN INTACT
O2 Administration : No
Remarks : ON ROOM AIR
SPO2(Latest reading in the shift) : 98
DIET : NPO
On tube feeding : No
IV Fluids : No
Blood/Blood products : No
Total Parenteral Nutrition : No
Pre Antibiotic Culture : No
Invasive Lines : No
Self Void : Yes
Remarks : 1 TIME
Foley's : No
Drain : No
Bowel Opened : No
Is the patient posted for any Surgery / Procedure : No
b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
Dressing Present : No
High Risk medications : No
Is Patient on Physiotherapy : No
Ambulation : Ambulant



Hospital No: 67046832

Visit No:

Name:

Doctor Name:

Investigations Sent : No
 Reports Collected : No
 Care Plan : Yes
 Remarks : PAIN AND FALL AND NUTRITION
 Special Needs : No
 Patient & Family Education : No
 Transfer : No
Medications: No
 Planned Procedure : No
 Approved follow-up : No
 Transfusion : No
 Patient belongings with health care providers : No
 Pending Reports : No
 Critical Value : No
 Pharmacy / Consumables : Charged
 Remarks : GLOVES
 Discharge Plan : No
 Comments : PATIENT SHIFTED TO 4TH OT WITH IP AND OP FILE.
 Hand over by : Raseena P P
 Handed over to : Miss Kaveri K.S
 Team Lead Hand over By : Josna Joseph
 Team Lead Hand over To : Saranya Nagarajan
 Status : Entered
 Pre-Op Check List(N)
 Status : Entered
 Score : 0
 ID Band correct and in place : Yes
 Valid consent signed : Yes
 Premed given and recorded : No
 Nursing Discharge Summary Completed : On PACS
 Procedural area prepared/marked : No
 Prosthesis/Hearing aid : No
 Dentures removed/insitu : Insitu
 Hair clips/make up/nail varnish removed : Yes
 Last food : 23/03/2026
 Last voided : 24/03/2026
 Consent incomplete action taken : Surgeon notified
 Jewellery removed/taped : Yes
 Contact lenses/glasses removed : No
 Last fluid : 23/03/2026
 TEDS/Calf compressors : N/A
 Multi resistant organism : No
 CJD Assessment : No
 ECG Attended : No
 Blood results available : Yes
 Blood available : No
 Valid blood consent signed : No



Hospital No: 67046832	Visit No:
Name:	Doctor Name:

Allergies recorded : No
 Observation Group
 Pre-OP Check List Last Administered: 24/03/2026 11:53

 Ward Check
 OT Check
 Shaved : Yes
 Bowels last opened : 24/03/2026
 Patient's Xray available
 Antibiotic prescribed : No
 Antibiotic given : No
 Antithrombotic prescribed : No
 Antithrombotic given : No
 Has a pregnancy test been performed? : No
 Is the patient pregnant? : No
 Pacemaker : No
 Defibrillator : No
 Suitable clothing : Yes
 Is clotting normal : No
 Anti-coagulant checked : Yes
 Platelets checked : Yes
 Group and Save requested : No
 Cross match : No
 Routine medication : No
 Gowned, underwear removed : Yes
 Anti-embolism stockings on patient and correctly fitted : No
 Are there any pressure areas or areas of frail skin need to be considered when positioning patient : No
 Nursing / Care assessment
 Additional information (Anxiety, Dementia, Mobility, Communication, Pre-existing medical conditions)
 Care Provider Name : Raseena P P
 Care Provider Role : NURSE
 OT Nurse Name : Saranya Nagarajan
 OT Nurse Role : NURSE
 Ward Nurse
 OT Nurse
 Team Lead Hand over By : Josna Joseph
 Team Lead Hand over To : Saranya Nagarajan
 OT MOVEMENTS
 Status : Entered
 Score : 0
 Operation Date : 24/03/2026
 Area in Date : 24/03/2026
 Area in Time : 12:00
 Theatre In Date : 24/03/2026
 Theatre In Time : 13:20
 Theatre Out Date : 24/03/2026



Hospital No: 67046832

Visit No:

Name:

Doctor Name:

Theatre Out Time : 15:15
 Area Out Date : 24/03/2026
 Area Out Time : 15:20
 Shifting to ICU : Y
 Area In update user : Triveni T
 Date & Time : 24/03/2026 15:04
 Theatre In Update user : Triveni T
 Date & time : 24/03/2026 15:04
 Theatre out update user : Triveni T
 Date & time : 24/03/2026 15:23
 Area Out update user : Triveni T
 Date & Time : 24/03/2026 15:23
 Area In
 Theatre In
 Theatre Out
 Area Out
 Theatre Location : MHM 4F OT 2
 Incision Time : 01:45
 Closure Time : 14:50
 Incision Date : 24/03/2026
 Closure Date : 24/03/2026
 Incision Time Updated User : Triveni T
 IT Updated Date and Time : 24/03/2026 15:05
 CT Updated User Name : Triveni T
 CT Updated Date and Time : 24/03/2026 15:05
 Surgery cat : Elective
 Shower with chlorhexidine : YES
 Additional dose of antibiotic given if surgery continues for > 4hrs : NO
 Hair removed with clipper on the surgery site : YES
 DM : NO
 Scheduled Start Time : 11:30
 Scheduled Date : 24/03/2026
 Antibiotic
 Antibiotic name : inj supacef
 750mg
 Date :
 24/03/2026
 Time : 13:30

 Wound Classification : Clean
 Scrub Staff : SARANYA
 Floor Staff : TRIVENI
 Anesthesia Technician : PRABHAKAR
 Shifting Staff : TRIVENI
 Site prepared with povidone Iodine or 2% chlorhexedine gluconate: : YES
 Has the antiseptic site preparation dried before skin incision : YES
 Ward Updated User : Raseena P P



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

Ward Date & Time : 24/03/2026 11:54
Intra Updated User : Vivek Raj V
Intra Date & Time : 24/03/2026 20:05
Patient/Family Health Education
Status : Entered
Date: : 24/03/2026
Time: : 09:38
Assessment Type : Initial Assessment

Educational Needs : Disease process, Safe and Effective Use of Medication, side effects and Interactions, Safe and effective use of medical equipment, Pain Management, Patients Rights and Responsibilities, Personal hygiene and practitioner hand wash, Care plan and treatment, Informed consent procedure, Activity/Exercise, Diagnostic Test/ Procedure, Health Maintenance, Safety, Falling precaution, Hand hygiene
Given Education : health education explained about falling, diet, hand hygiene
Teaching Method : One to One
Teaching Tool : Verbal Instructions
Evaluation : Applies knowledge/skills
Child Attends School : Not Applicable
Does the child has any problem at school : Not Applicable
Literacy Skills : Yes
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to improve the nutritional intake Nursing Diagnosis : Imbalance nutrition less than body requirement due to intolerance of diet.
Education and comments: : Provide technique to maintain or restore adequate nutritional intake to stimulate appetite.
Nursing Interventions : Monitor patient dietary history, pattern of indigestion and intolerance of food.
- Provide technique to maintain or restore adequate nutritional intake to stimulate appetite.
- Maintain pleasant environment for the patient.
Evaluation : Continue
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : To reduce risk for fall Nursing Diagnosis : Risk for fall
Education and comments: : Provide education on how to use call bell and observe the patient demonstration
Nursing Interventions : a) Orient to surroundings on admission.
b) Provide education on how to use call bell and observe the patient demonstration
c) Provide appropriate safety measures like sitter, display vulnerable patient alert.
d) Lower the height of bed, keep the patient near nurses station
e) Provide education on safety measures to patient and attender.
Evaluation : Continue
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes
Nursing Care Plan discussed with Family : Yes



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Name:

Visit No:
Doctor Name:

Patient Measurable Goal for the day : To reduce the pain Nursing Diagnosis : Acute pain ./ or post-operative pain
Education and comments: : Eliminate factors that precipitate pain.

Nursing Interventions : * Assess the condition of the patient

*Eliminate factors that precipitate pain.

* Monitor characteristics of pain (Location, severity using appropriate pain scale), frequency, precipitating factors and relief factors.

* followed with physician orders

* Provide restful and quiet environment

Evaluation : Continue

HOURLY NURSING ROUND

Score : 0

HOURLY NURSING ROUND - DAY

CHECKPOINTS

Introduce yourself : 10 AM,07 PM

Ask for pain if any : 10 AM,11 AM,12 NOON,07 PM,08 PM

Ask for Comfort or Position : 10 AM,11 AM,12 NOON,07 PM

Ask for bathroom needs : 10 AM,11 AM,12 NOON,07 PM,08 PM

Peripery-move the phone, Call Light, Trashcan, Watercan, Over-Bed Table : 10 AM,11 AM,12 NOON,07 PM,08 PM

Pump-Syringe pump, Infusion Line, IV Line : 10 AM,07 PM

HOURLY NURSING ROUND - NIGHT

CHECKPOINTS

Check for Sleep : 11 PM,01 AM,03 AM,05 AM

Comments : @ 9.20 AM

- RECIEVED PATIENT FROM ADMISSION DESK

@ 12 PM PATIENT SHIFTED TO 4TH OT

Ask for pain if any : 09 PM,07 AM

Ask for Comfort or Position : 11 PM,01 AM,03 AM,07 AM

Ask for bathroom needs : 09 PM,05 AM

Peripery-move the phone, Call Light, Trashcan, Watercan, Over-Bed Table : 09 PM,07 AM

Pump-Syringe pump, Infusion Line, IV Line : 11 PM,01 AM,03 AM,05 AM,07 AM

Surgical Site Infection Prevention Bundle

Status : Entered

Booked procedure : ADENOTONSILLECTOMY (LENT)

Name of surgeon : DR.SAUD AHMED

Actual procedure : ADENOTONSILLECTOMY (LENT)

Theatre No: : 02

Date of procedure : 24/03/2026

Time of procedure: From : 13:45

To : 14:50

Pre-operative chlorhexidine bath supplied : Yes

Bath taken night prior to surgery : Yes

Bath taken on the day of surgery : Yes

Prophylactic antibiotic prescribed : Yes

Type of hair removal : Other



Hospital No: 67046832

Visit No:

Name:

Doctor Name:

Antibiotic prophylaxis given : Yes
Given in : OT
Skin antisepsis done with : Providone-Iodine
Type of surgery : Clean
Maintaining Normothermia (>36 C) : Yes

Forms

OT Checklist & Instrument Count
Score : 0
OT CHECKLIST AND INSTRUMENT COUNT
SCRUB NURSE : Saranya Nagarajan
SURGEON : DR.SAUD AHMED
CIRCULATING NURSE : Triveni T
ASSISTANT : Dr. Indresh Chandra
ANESTHETIST : Dr. Jadhav Tanmay Ashok
DATE : 24/03/2026
OT NUMBER : 02
NEEDLES, BLADES & SWABS
ITEM
INITIAL/ START COUNT
CAVITY/ SECOND COUNT
CLOSING/ THIRD COUNT
ADDED COUNT
FINAL COUNT
BP Blade : 1
BP Blade : 1
BP Blade : 1
BP Blade : 1
Gauze 4x4 : 10
Gauze 4x4 : 10
Gauze 4x4 : 10
Gauze 4x4 : 10
INSTRUMENTS
INSTRUMENT
INITIAL/ START COUNT
CAVITY/ SECOND COUNT
CLOSING/ THIRD COUNT
ADDED COUNT
FINAL COUNT
Mosquito curved : 2
Mosquito curved : 2
Mosquito curved : 2
Mosquito curved : 2
Allis 6 inch : 1
Allis 6 inch : 1
Allis 6 inch : 1
Allis 6 inch : 1
Scissors Curved : 1



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Doctor Name:

Scissors Curved : 1
Scissors Curved : 1
Scissors Curved : 1
Scissors straight : 1
Scissors straight : 1
Scissors straight : 1
Scissors straight : 1
Toothed 4 inch : 1
Toothed 4 inch : 1
Toothed 4 inch : 1
Toothed 4 inch : 1
BP Handle 7 inch : 1
BP Handle 7 inch : 1
BP Handle 7 inch : 1
BP Handle 7 inch : 1

COUNT STATUS AND DISCREPANCY RESOLUTION

Scrub Nurse : Saranya Nagarajan

Circulating Nurse : Triveni T

Surgical Safety Checklist(New)

Status : Entered

Score : 0

Before Induction of Anaesthesia

Sign in : 24/03/2026

Identity : Y

Procedure : Y

Consent : Y

Site Marked : Not Applicable

Pulse oximeter on patient and functioning : Yes

Essential equipment/assistance available : Yes

Does patient have known allergy? : No

Difficult airway/aspiration risk? : No

Risk Of >500ML blood loss (7ml/kg in children)? : No

Before skin incision

Time Out : 24/03/2026

Confirm all Team members have introduced themselves by Name and Role : Y

Correct Patient : Y

Correct Site : Y

Correct Procedure : Y

Has antibiotic prophylaxis been given within the last 60 minutes? : Yes

Essential imaging is available : Yes

Before patient leaves operation room

Sign Out : 24/03/2026

The Nurse has confirmed the name of the procedure (s) with the Surgeon and Anesthetist : Yes

All items counts are correct : Yes

Specimen(s) is/are correctly labelled (including Patient URN): : Not Applicable

Are there any equipment issues to be addressed? : No



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Name:

Visit No:
Doctor Name:

Surgeon, Anesthetist and Nurse have reviewed any and all key concerns for the recovery and management of the Patient : Yes
Sign In Time : 13:25
Time Out Time : 13:45
Surgical team review:
Are there any equipment issues or concerns to be addressed : No
Sign Out time : 14:50
Are there any patient-specific concerns? : No
Nursing Team Review:
Site : Y
Anesthesia safety check completed : Y
Sign In Nurse : Triveni T
Correct Position : Y
Critical or non routine steps : ROUTINE STEPS
Operation duration (min) : 2 HOURS
Has sterility (including indicator result) been confirmed? : Yes
Are there any equipment issues or concerns? : No
Time Out Nurse : Triveni T
Sign Out Nurse : Triveni T
Anaesthesia Team Review:
Time Out Surgeon : DR.SAUD AHMED
Time Out Anesthetist : Dr. Jadhav Tanmay Ashok
Pre-Op Check List(N)
Status : Entered
Score : 0
ID Band correct and in place OT : Yes
Valid consent signed OT : Yes
Premed given and recorded OT : No
X-Rays OT : Films with Pat.
Procedural area prepared/marked OT : Yes
Prosthesis/Hearing aid OT : N/A
Dentures OT : Own teeth
Hair clips/ make up/ nail varnish removed OT : Yes
Jewelery removed/taped OT : Yes
Contact lenses/glasses removed OT : N/A
TEDS/Calf compressors OT : N/A
Multi resistant organism OT : No
CJD Assessment OT : No
Blood results available OT : Yes
Blood available OT : No
Allergies recorded : NKA
Observation Group
Pre-OP Check List Last Administered: 24/03/2026 15:06

Ward Check
OT Check



Hospital No: 67046832

Visit No:

Name:

Doctor Name:

Shaved OP : Yes
 Patient's Xray available
 Antibiotic prescribed OT : Yes
 Antibiotic given OT : Yes
 Pacemaker OT : N/A
 Defibrillator OT : N/A
 Suitable clothing OT : Yes
 Nursing / Care assessment
 Additional information (Anxiety, Dementia, Mobility, Communication, Pre-existing medical conditions)
 OT Nurse Name : Triveni T
 Ward Nurse
 OT Nurse
 Team Lead Hand over To : Mohini M K

Alerts

Peripheral Line1 - Onset: 24/03/2026
 PERIPHERAL LINE NO 22G IN LEFT HAND

Forms

Skin Examination
 Pallor : No
 Cyanosis : No
 Skin turgor : Normal turgor
 Jaundice : No
 Erythema : No
 Petechiae : No
 Purpura : No
 Oedema : No
 Rash : No
 Skin Lesions : No
 Surgical scars : No
 Wounds : No
 Burns : No
 Hair : Normal

Nails : Normal

Pain Assessments

Score: 1
 Type of Assessment: Shift Assessment
 Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
 Numeric Pain Scale: 1

Additional Pain Assessment Details



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Since: 24/03/2026
Type: Stabbing
Duration: Irregular
Continuous: Constant
Time: At rest
Onset: Gradual

Associated Symptoms: None
Comments: INJ PCT 1GM GIVEN

Intravascular Access Lines
Status : Entered
IntraVascular Access Type : Peripheral IV Line
Peripheral IV Activity : New insertion
Peripheral IV Insertion Date : 24/03/2026
Peripheral IV Insertion Time : 13:00
Peripheral IV Catheter Type : IV Cannula
Peripheral IV Location : Left
Peripheral IV Site : Hand
Peripheral IV Gauge : 22 Gauge
Peripheral IV Procedure Preparation : Handwashing observed,Hand hygiene performed
Peripheral IV Insertion Indication : IV hydration / medication
Peripheral IV Procedure Result : Successful insertion
Peripheral IV Number Of Attempts : 1
Peripheral IV Equipment used : Manual,Extension set
Referred To IV Nurse : No
CVC Site Assesst

Lumen color : BLUE
Central IV TPA Intervention

Catheter position corrected / withdrawn

Arterial Line Status

Peripheral IV Assessment
Need Assessment :
Yes
Care : Cap(s)
changed
Site Details : No
complications
Dressing Type / Appearance :
Dry,Transparent
Peripheral IV Patency : No



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complications
Need Assessment :
Yes
Care : Secured with
tape
Site Details : No
complications
Dressing Type / Appearance :
Dry,Intact
Peripheral IV Patency : No
complications
Need Assessment :
Yes
Site Details : No
complications
Dressing Type / Appearance :
Dry,Intact,Transparent
Peripheral IV Patency : No complications

Forms

Nurses Endorsment Checklist
Shift : Transfers
Situation : Elective,Surgery,NON MLC,Transfer Out
Comment / Chief Complaints / Diagnosis : POST OF ADENOTONSILECTOMY POD-00
Background
Allergies : No
Remarks : UNKNOWN
Diet : Yes
Remarks : NPO
Previous Medications : No
Remarks : NIL
Old reports added to TRAK : No
Remarks : NIL
Consents : Yes
Remarks : SURGERY
Cross Consultation : No
Remarks : NIL
Procedures performed before transfer : No
Remarks : NIL
Vulnerable : Yes
Remarks : AGE BELOW 15 YEARS
Past Medical / Surgical History : NIL
Intubation : N/A
Ventilation Status : N/A
ET Size and Lip Level : N/A
Tracheostomy : N/A
ICD : N/A



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Visit No:

Name:

Doctor Name:

Pig Tail : N/A
 T Piece : N/A
 Chest Physio : N/A
 Treated with Inotropes : N/A
 Blood Transfusion : N/A
 Level of Consciousness : Oriented
 GCS : E4M6V5
 Vitals -TPR & BP : is stable : Yes
 Pain Score : 2
 On MET/PET List : Yes
 Risk For FALL : No
 Risk For Pressure Ulcer : No
 Risk for Aspiration : No
 Existence of Pressure Ulcer : No
 O2 Administration : No
 SPO2(Latest reading in the shift) : 98
 DIET : Oral
 On tube feeding : No
 IV Fluids : No
 Blood/Blood products : No
 Total Parenteral Nutrition : No
 Pre Antibiotic Culture : No
 Invasive Lines : No
 Self Void : No
 Foley's : No
 Drain : No
 Bowel Opened : No
 Is the patient posted for any Surgery / Procedure : No
 b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
 Dressing Present : No
 High Risk medications : No
 Is Patient on Physiotherapy : No
 Ambulation : Ambulant
 Investigations Sent : No
 Reports Collected : No
 Care Plan : No
 Pupil Reaction : 2MM
 Inotropes : No
 Ventilator : No
 Dialysis : No
 Special Needs : No
 Patient & Family Education : No
 Transfer : No
Medications: No
 Planned Procedure : No
 Approved follow-up : No
 Transfusion : No



Hospital No: 67046832

Visit No:

Name:

Doctor Name:

Patient belongings with health care providers : No
Pending Reports : No
Critical Value : No
Pharmacy / Consumables : Charged
Remarks : Consumables
Discharge Plan : No
Comments : NPO TILL 7PM.
Hand over by : Ranjitha T U
Handed over to : Navami Jayaprakash
Team Lead Hand over By : Ranjitha T U
Team Lead Hand over To : Shreedevi C

Forms

Skin Examination

Pallor : No

Cyanosis : No

Jaundice : No

Erythema : No

Petechiae : No

Oedema : No

Rash : No

Skin Lesions : No

Surgical scars : No

Wounds : No

Burns : No

Hair : Normal

Nails : Normal

Patient Acuity Tool

Stable patient 1 to 11

Moderate Risk Patient 12 to 22

Complex Patient 22 to 33

High Risk Patient >34

Total Score :16

Risk Assessment - Fall Paediatric (Humpty Dumpty)

Status : Entered

Score : 8 Low Risk Humpty Dumpty Score

Indication for assessment : Reassessment

Age : 13 years old and above - 1

Gender : Female - 1

Diagnosis:

Other diagnosis - 1

Cognitive Impairment : Orientated to own ability - 1



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

Environmental Factors : Patient placed in bed - 2
Response to Surgery/Sedation/Anaesthesia : More than 48 hours / None - 1
Medication Usage : Other medications / None - 1
Paediatric Braden Q Scale MHE
Status : Entered
Score : 20
SENSORY PERCEPTION - Ability to respond meaningfully to pressure-related discomfort : 4 - NO IMPAIRMENT
– Responds to verbal commands. Has no sensory deficit which would limit ability to feel or voice pain or discomfort
MOISTURE - Degree to which skin is exposed to moisture : 4 - RARELY MOIST – Skin is usually dry; linen only
requires changing at routine intervals
ACTIVITY - Degree of physical activity : 4 - WALKS FREQUENTLY– Walks outside the room at least twice a day
and inside room at least once every 2 hours during waking hours
MOBILITY - Ability to change and control body position : 4 - NO LIMITATIONS – Makes major and frequent
changes in position without assistance
NUTRITION - Usual food intake pattern : 4 - EXCELLENT – Eats most of every meal. Never refuses a meal.
Usually eats a total of 4 or more servings of meat and dairy products. Occasionally eats between meals. Does not
require supplementation
FRICTION AND SHEAR : 4 - NO APPARENT PROBLEM – Moves in bed and in chair independently and has
sufficient muscle strength to lift up completely during move. Maintains good position in bed or chair at all times
13 - 15: Moderate risk
The Braden scale assesses a patients risk of developing a pressure ulcer by examining six criteria. with a higher score
meaning a lower risk of developing a pressure ulcer and vice-versa.
9 or less: Very High Risk
10 - 12: High Risk
15 - 18: Low Risk
16 - 23: No Risk
Total Score : 28
TISSUE PERFUSION AND OXYGENATION : 4-Excellent:Normotensive:Oxygen saturation may be > 95%,
normal heamoglobin and capillary refill < 2 secs
Type of Assessment : Shift Assessment
ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL
Score : 0
CATEGORIES

TOTAL SCORE : 0
COMMENTS : low risk

Forms

Patient/Family Health Education
Status : Entered
Date : 24/03/2026
Time : 22:11

Educational Needs : Safe and Effective Use of Medication, side effects and Interactions, Safe and effective use
of medcial equipment, Pain Management, Pesonal hygiene and practitioner hand wash, Infection control related
issues, Informed consent procedure, Activity/Exercise, Nutrition, Risk, Safety, Lines/drain/catheter reconciliation, Falling
precaution, Medication side effects
Given Education : education given regarding diet and exercise



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

Teaching Method : Other
Child Attends School : No
Does the child has any problem at school : No
Literacy Skills : No
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to prevent fall Nursing Diagnosis : Risk for fall
Education and comments: : Provide education on how to use call bell and observe the patient demonstration.
Nursing Interventions : a)Orient to surroundings on admission.
b)Provide education on how to use call bell and observe the patient demonstration.
c)Provide appropriate safety measures like sitter, display vulnerable patient alert.
d) Lower the height of bed, keep the patient near nurses station
e)Provide education on safety measures to patient and attender.
Evaluation : Continue
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to reduce the pain Nursing Diagnosis : Acute pain ./ or post-operative pain
Education and comments: : Eliminate factors that precipitate pain.
Nursing Interventions : * Assess the condition of the patient
* Monitor characteristics of pain (Location, severity using appropriate pain scale), frequency, precipitating factors and relief factors.
* Eliminate factors that precipitate pain.
* Other analgesic 6th/8th hourly if necessary in accordance with physician orders
* Provide restful and quiet environment
Evaluation : Continue

Forms

Nurses Endorsment Checklist
Status : Entered
Shift : Night to Morning
Situation : Elective,Surgery,NON MLC,Transfer Out
Comment / Chief Complaints / Diagnosis : POST OF ADENOTONSILECTOMY POD-00
Background
Allergies : No
Remarks : UNKNOWN
Diet : Yes
Remarks : NPO
Previous Medications : No
Remarks : NIL
Old reports added to TRAK : No
Remarks : NIL
Consents : Yes
Remarks : SURGERY
Cross Consultation : No



Hospital No: 67046832

Visit No:

Name:

Doctor Name:

Remarks : NIL
Procedures performed before transfer : No
Remarks : NIL
Vulnerable : Yes
Remarks : AGE BELOW 15 YEARS
Past Medical / Surgical History : NIL
Level of Consciousness : Oriented
GCS : 15/15
Vitals -TPR & BP : is stable : Yes
Pain Score : 02
On MET/PET List : No
Risk For FALL : Yes
Remarks : 8 low risk
Risk For Pressure Ulcer : No
Risk for Aspiration : Yes
Remarks : 0 low risk
Existence of Pressure Ulcer : No
O2 Administration : No
SPO2(Latest reading in the shift) : 98
DIET : Oral
Diet-Specify the type : Normal
On tube feeding : No
IV Fluids : No
Blood/Blood products : No
Total Parenteral Nutrition : No
Pre Antibiotic Culture : No
Invasive Lines : Yes
Remarks : 22g on right hand
Self Void : Yes
Remarks : 3 times
Foley's : No
Drain : No
Bowel Opened : No
b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
Dressing Present : No
High Risk medications : No
Is Patient on Physiotherapy : No
Ambulation : Ambulant
Investigations Sent : No
Reports Collected : No
Care Plan : Yes
Remarks : pain,fall risk and nutrition
Special Needs : No
Patient & Family Education : No
Transfer : No
Medications: Yes
Remarks : inj.paracetamol 400mg sos



Hospital No: 67046832	Visit No:
Name:	Doctor Name:

Planned Procedure : No
 Approved follow-up : No
 Transfusion : No
 Patient belongings with health care providers : No
 Pending Reports : No
 Critical Value : No
 Pharmacy / Consumables : Charged
 Remarks : consumables
 Discharge Plan : No
 Comments : patient is stable
 Hand over by : Navami Jayaprakash
 Handed over to : Ankita Biswas
 Team Lead Hand over By : Bhagyashri cholappanavar
 Team Lead Hand over To : Angelin Jenifer
 Comments by Team Lead : patient is stable
 Status : Entered

Forms

Skin Examination
 Pallor : No
 Cyanosis : No
 Jaundice : No
 Erythema : No
 Petechiae : No
 Oedema : No
 Rash : No
 Skin Lesions : No
 Surgical scars : No
 Wounds : No
 Burns : No
 Hair : Normal

Nails : Normal

Pain Assessments
 Score: 2
 Type of Assessment: Shift Assessment
 Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
 Numeric Pain Scale: 2

Additional Pain Assessment Details

Type: Dull,Pulling



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

Duration: Irregular
Continuous: Variable
Time: Not Applicable
Onset: Gradual

Associated Symptoms: None

Patient Acuity Tool
Stable patient 1 to 11
Moderate Risk Patient 12 to 22
Complex Patient 22 to 33
High Risk Patient >34

Total Score :7
Paediatric Fall-Humpty Dumpty
Status : Entered
Score : 8
Age : 13 years old and above - 1
Gender : Female - 1

Diagnosis:

Other diagnosis - 1
Cognitive impairment : Orientated to own ability - 1
Environmental factors : Patient placed in bed - 2
Response to surgery / sedation / anaesthesia : More than 48 hours / None - 1
Medication usage : Other medications / None - 1
7 - 11: Low humpty dumpty score
>=12: High risk humpty humpty score
Allows care providers to rapidly and easily assess child's likelihood of falling.

7 - 11
Low humpty dumpty score
#12
High risk humpty dumpty score
Score

Classification
Total Score : 9
Type of Assessment : Shift Assessment
Paediatric Braden Q Scale MHE
Status : Entered
Score : 20

SENSORY PERCEPTION - Ability to respond meaningfully to pressure-related discomfort : 4 - NO IMPAIRMENT
- Responds to verbal commands. Has no sensory deficit which would limit ability to feel or voice pain or discomfort
MOISTURE - Degree to which skin is exposed to moisture : 4 - RARELY MOIST - Skin is usually dry; linen only
requires changing at routine intervals



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

ACTIVITY - Degree of physical activity : 4 - WALKS FREQUENTLY– Walks outside the room at least twice a day and inside room at least once every 2 hours during waking hours
MOBILITY - Ability to change and control body position : 4 - NO LIMITATIONS – Makes major and frequent changes in position without assistance
NUTRITION - Usual food intake pattern : 4 - EXCELLENT – Eats most of every meal. Never refuses a meal. Usually eats a total of 4 or more servings of meat and dairy products. Occasionally eats between meals. Does not require supplementation
FRICITION AND SHEAR : 4 - NO APPARENT PROBLEM – Moves in bed and in chair independently and has sufficient muscle strength to lift up completely during move. Maintains good position in bed or chair at all times
 13 - 15: Moderate risk
 The Braden scale assesses a patients risk of developing a pressure ulcer by examining six criteria. with a higher score meaning a lower risk of developing a pressure ulcer and vice-versa.
 9 or less: Very High Risk
 10 - 12: High Risk
 15 - 18: Low Risk
 16 - 23: No Risk
 Total Score : 28
TISSUE PERFUSION AND OXYGENATION : 4-Excellent:Normotensive:Oxygen saturation may be > 95%, normal heamoglobin and capillary refill < 2 secs
 Type of Assessment : Shift Assessment

Forms

Nurses Endorsment Checklist
 Status : Entered
 Shift : Morning to Evening
 Situation : Elective,NON MLC,Shift Handover
 Comment / Chief Complaints / Diagnosis : POST OF ADENOTONSILECTOMY POD-00
 Background
 Allergies : No
 Remarks : UNKNOWN
 Diet : Yes
 Remarks : normal diet
 Previous Medications : No
 Remarks : NIL
 Old reports added to TRAK : No
 Remarks : NIL
 Consents : Yes
 Remarks : SURGERY
 Cross Consultation : No
 Remarks : NIL
 Procedures performed before transfer : No
 Remarks : NIL
 Vulnerable : Yes
 Remarks : AGE BELOW 15 YEARS
 Past Medical / Surgical History : NIL
 Level of Consciousness : Oriented
 GCS : 15/15
 Vitals -TPR & BP : is stable : Yes



Hospital No: 67046832

Visit No:

Name:

Doctor Name:

Pain Score : 2
 On MET/PET List : No
 Risk For FALL : No
 Risk For Pressure Ulcer : No
 Risk for Aspiration : No
 Existence of Pressure Ulcer : No
 Remarks : SKIN INTACT
 O2 Administration : No
 Remarks : ON ROOM AIR
 SPO2(Latest reading in the shift) : 98
 DIET : Oral
 Diet-Specify the type : Normal
 On tube feeding : No
 IV Fluids : No
 Blood/Blood products : No
 Total Parenteral Nutrition : No
 Pre Antibiotic Culture : No
 Invasive Lines : Yes
 Remarks : 22G LEFT HAND DAY-1
 Self Void : Yes
 Remarks : 3 TIMES
 Foley's : No
 Drain : No
 Bowel Opened : No
 Is the patient posted for any Surgery / Procedure : No
 b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
 Dressing Present : No
 High Risk medications : No
 Is Patient on Physiotherapy : No
 Ambulation : Ambulant
 Investigations Sent : No
 Reports Collected : No
 Care Plan : Yes
 Remarks : PAIN AND FALL AND NUTRITION
 Special Needs : No
 Patient & Family Education : No
 Transfer : No
Medications: Yes
 Remarks : TAB:TAXIM O BD
 Planned Procedure : No
 Approved follow-up : No
 Transfusion : No
 Patient belonging with health care providers : No
 Pending Reports : No
 Critical Value : No
 Pharmacy / Consumables : Charged
 Remarks : GLOVES



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

Discharge Plan : Yes
Date of Discharge : 25/03/2026
Comments : DISCHARGE WAITING FOR CLEARANCE
Hand over by : Ankita Biswas
Handed over to : Shreedevi C
Team Lead Hand over By : Angelin Jenifer
Team Lead Hand over To : Shreedevi C
Status : Entered
HOURLY NURSING ROUND
Score : 0
HOURLY NURSING ROUND - DAY
CHECKPOINTS
Introduce yourself : 08 AM
Ask for pain if any : 08 AM,10 AM
Ask for Comfort or Position : 08 AM,09 AM,11 AM
Ask for bathroom needs : 08 AM,10 AM
Peripery-move the phone, Call Light, Trashcan, Watercan, Over-Bed Table : 08 AM,09 AM,11 AM
Pump-Syringe pump, Infusion Line, IV Line : 08 AM,10 AM
HOURLY NURSING ROUND - NIGHT
CHECKPOINTS
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : provide nutritional support Nursing Diagnosis : Imbalance nutrition less than body requirement due to intolerance of diet.
Education and comments: : to provide good nutritional level.
Nursing Interventions : Maintained pleasant environment for the patient.
Nursing Interventions : - Monitor patient dietary history, pattern of indigestion and intolerance of food.
- Provide technique to maintain or restore adequate nutritional intake to stimulate appetite.
- Maintain pleasant environment for the patient.
Evaluation : Terminate
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : provide side rails Nursing Diagnosis : Risk for fall
Education and comments: : education given regarding fall risk
Nursing Interventions : a)Orient to surroundings on admission.
b)Provide education on how to use call bell and observe the patient demonstration.
c)Provide appropriate safety measures like sitter, display vulnerable patient alert.
d) Lower the height of bed, keep the patient near nurses station
e)Provide education on safety measures to patient and attender.
Evaluation : Terminate
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : to reduce the pain Nursing Diagnosis : Acute pain ./ or post-operative pain
Education and comments: : education given regarding pain management
Nursing Interventions : * Assess the condition of the patient
* Monitor characteristics of pain (Location, severity using appropriate pain scale), frequency, precipitating factors and relief factors.
* Eliminate factors that precipitate pain.
* Other analgesic 6th/8th hourly if necessary in accordance with physician orders
* Provide restful and quiet environment
Evaluation : Terminate
Patient/Family Health Education
Status : Entered
Date: : 25/03/2026
Time: : 09:45
Assessment Type : Other1

Educational Needs : Disease process, Safe and Effective Use of Medication, side effects and Interactions, Pain Management, Personal hygiene and practitioner hand wash, Care plan and treatment, Home Instruction, Activity/ Exercise, Nutrition, Risk, Safety, Falling precaution, Hand hygiene
Given Education : health education about hand hygiene
Teaching Method : One to One
Teaching Tool : Verbal Instructions
Evaluation : Verbalize basic concept with assistance
Child Attends School : No
Does the child has any problem at school : No
Literacy Skills : No

Diet Notes :
THE PATIENT IS ON SOFT DIET
ENERGY-1500KCAL
PROTEIN-45G

Initial Nutritional Assessment
Status : Authorised
ANTHROPOMETRIC MEASUREMENTS
Height (cms) : 138
Weight (kgs) : 27.9
IBW (kgs) : 43.61
BMI : 14.18
Recent weight loss/gain : None
GI SYMPTOMS (Recent)
Food allergies/Intolerance : No
Subjective Global Assessment rating
Nutritional intervention proposed/Diet advised : Soft diet, diet progression as per medical condition
Others : Clinical notes will be updated on change of diet
Select Type : Assessment
Evaluation Criteria



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

Adult Others:ADENOTONSILLITIS

Forms

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 1
Remarks : low risk
Age : Below 60 Years - 0
Patient care Equipment : One Present- 1
Type of Assessment : Shift Assessment
Pain Assessments
Score: 2
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 2

Additional Pain Assessment Details

Since: 25/03/2026
Type: Dull,Not Applicable
Duration: Not Applicable
Time: Not Applicable

Associated Symptoms: None

Skin Examination
Pallor : No
Cyanosis : No
Jaundice : No
Erythema : No
Petechiae : No
Oedema : No
Rash : No
Skin Lesions : No
Surgical scars : No
Wounds : No
Burns : No
Hair : Normal



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

Nails : Normal

Paediatric Fall-Humpty Dumpty
Status : Entered
Score : 10
Age : 13 years old and above - 1
Gender : Female - 1

Diagnosis:

Other diagnosis - 1
Cognitive impairment : Orientated to own ability - 1
Environmental factors : Patient placed in bed - 2
Response to surgery / sedation / anaesthesia : Within 24 hours - 3
Medication usage : Other medications / None - 1
7 - 11: Low humpty dumpty score
≥12: High risk humpty dumpty score
Allows care providers to rapidly and easily assess child's likelihood of falling.

7 - 11

Low humpty dumpty score

#12

High risk humpty dumpty score

Score

Classification

Total Score : 11

Type of Assessment : Shift Assessment

Paediatric Braden Q Scale MHE

Status : Entered

Score : 20

SENSORY PERCEPTION - Ability to respond meaningfully to pressure-related discomfort : 4 - NO IMPAIRMENT
– Responds to verbal commands. Has no sensory deficit which would limit ability to feel or voice pain or discomfort

MOISTURE - Degree to which skin is exposed to moisture : 4 - RARELY MOIST – Skin is usually dry; linen only
requires changing at routine intervals

ACTIVITY - Degree of physical activity : 4 - WALKS FREQUENTLY– Walks outside the room at least twice a day
and inside room at least once every 2 hours during waking hours

MOBILITY - Ability to change and control body position : 4 - NO LIMITATIONS – Makes major and frequent
changes in position without assistance

NUTRITION - Usual food intake pattern : 4 - EXCELLENT – Eats most of every meal. Never refuses a meal.

Usually eats a total of 4 or more servings of meat and dairy products. Occasionally eats between meals. Does not
require supplementation

FRICTION AND SHEAR : 4 - NO APPARENT PROBLEM – Moves in bed and in chair independently and has
sufficient muscle strength to lift up completely during move. Maintains good position in bed or chair at all times

13 - 15: Moderate risk

The Braden scale assesses a patient's risk of developing a pressure ulcer by examining six criteria. with a higher score
meaning a lower risk of developing a pressure ulcer and vice-versa.

9 or less: Very High Risk



Hospital No: 67046832
Name:

Visit No:
Doctor Name:

10 - 12: High Risk

15 - 18: Low Risk

16 - 23: No Risk

Total Score : 28

TISSUE PERFUSION AND OXYGENATION : 4-Excellent:Nomotensive:Oxygen saturation may be > 95%,
normal heamoglobin and capillary refill < 2 secs

Type of Assessment : Shift Assessment

Forms

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : To improve the nutritional intake. Nursing Diagnosis : Imbalance nutrition
less than body requirement due to intolerance of diet.

Education and comments: : Education on diet.

Nursing Interventions : Monitor patient dietary history, pattern of indigestion and intolerance of food.

- Provide technique to maintain or restore adequate nutritional intake to stimulate appetite.

- Maintain pleasant environment for the patient.

Evaluation : Continue

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : To reduce the fall risk. Nursing Diagnosis : Risk for fall

Education and comments: : Prevent from fall.

Nursing Interventions : Orient to surroundings on admission.

b)Provide education on how to use call bell and observe the patient demonstration.

c)Provide appropriate safety measures like sitter, display vulnerable patient alert.

d) Lower the height of bed, keep the patient near nurses station

e)Provide education on safety measures to patient and attender.

Evaluation : Continue

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : To reduce the pain. Nursing Diagnosis : Acute pain ./ or post-operative pain

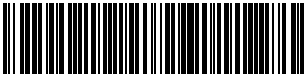
Education and comments: : Prevent from HAI.

Nursing Interventions : Provide pharmacologic pain management as per doctor's ,
administration of analgesic

Evaluation : Continue,Modify

MEDICATION ORDERS

Drug / Generic Item	Dosage Qty	Frequency	Instructions/Notes	Duration
OXYGEN MASK FLEXI CHILD, ROMSON	1 PC	ONCE		1 DAY(S)
UNDERPAD ABSORABLE MEDICAL(60 X90CM) ULTRACARE (UNDERPAD (PHARMACY NON-DRUG))	1 NUMBER	ONCE		1 DAY(S)
NORMAL SALINE 09% W/V AQUA PULSE 500 ML (SODIUM CHLORIDE (80009680))	1 BT	ONCE		1 DAY(S)



Hospital No: 67046832	Visit No:
Name:	Doctor Name:

MICRO POLYSYTE DUO 15CM 13341, POLYMED (SURGICALS (PHARMACY NON-DRUG))	1 PC	ONCE		1 DAY(S)
POLYFLUSH PRE-FILLED SYRINGE 5 ML # 90319, POLY MEDICURE LTD (SYRINGE (PHARMACY NON-DRUG))	2 PC	ONCE		1 DAY(S)
TAXIM-O 200 MG TABLET (CEFIXIME (80002486))	1 TAB	ONCE		1 DAY(S)
TAXIM-O DT 100 MG DISPERSIBLE TABLET (CEFIXIME (80002486))	1 TAB	12 HOURLY (Q12H)		1 DAY(S)
DESLOR 5 MG TABLET (DESLORATADINE 5 MG TABLET)	1 TAB	ONCE		1 DAY(S)
OTRIVIN PAEDIATRIC 0.05% W/V NASAL SPRAY 10 ML (XYLOMETAZOLINE~SORBITOL (80011655))	1 PC	ONCE		1 DAY(S)
BETADINE 2% W/V GARGLE 50 ML (POVIDONE-IODINE (80008203))	1 GARGLE	ONCE		1 DAY(S)
HYDROGEN PEROXIDE 20% W/V TOPICAL SOLUTION 100 ML (SURGICAL)	1 PC	ONCE		1 DAY(S)
IBUGESIC PLUS ORAL SUSPENSION 100 ML (IBUPROFEN~PARACETAMOL (80005395))	1 ORALSUSPENSION	STAT		
BED BATH TOWEL STERIX 320X320, BAPUJI (SURGICAL)	1 PC	ONCE		1 DAY(S)